

JOINT SYMPTOMS

Give urgent attention to the patient with a joint symptom/s and:

- Short history of single warm, swollen, extremely painful joint with limited range of movement, **septic arthritis** likely
- Injury in past 48 hours and severe pain/swelling or deformity, **fracture** likely →18.

- Temperature $\geq 38^{\circ}\text{C}$
- Unable to weight-bear

Management:

- If known gout and affected joint involves big toe, midfoot or ankle *and* no fever, wound, surgery or injection into joint, discuss with specialist if referral needed: if not, **acute gout** likely →152.
- Refer urgently.

Approach to the patient with joint symptoms not needing urgent attention

- Check joints and ask patient to place hands behind head, then behind back. Bury nails in palm and open hand. Press palms together with elbows lifted. Walk. Sit and stand up with arms folded.
- Is there any of: joint warm/tender/swollen or unable to do all actions comfortably?

No
Joint problem unlikely

- If body/general pain →60.
- If back pain →63.
- If neck pain →64.
- If arm symptoms →64.
- If leg symptoms →65.
- If foot symptoms →66.

Yes
Has there been recent injury?

No

Ask about duration of joint pain. Has joint pain lasted ≥ 6 weeks?

No

Has patient had recent genital discharge or painless non-itchy skin rash?

Yes

- Gonococcal arthritis** likely
- Usually involves wrists, ankles, hands and feet.
 - Refer/discuss same day.
 - Treat patient's partner/s as for cervicitis/male urethritis →49.

No

Any sudden onset of 1-3 warm, extremely painful, red, swollen joints (often big toe or knee)?

No

- Give **paracetamol** 1g 4-6 hourly (up to 4g in 24 hours) as needed for up to 5 days and **methyl salicylate ointment** to rub on affected joint. If no response, give **ibuprofen** 400mg 8 hourly with food as needed for up to 5 days (avoid if peptic ulcer, asthma, hypertension, heart failure, kidney disease).
- Test for HIV →110.
- Review after 1 month or sooner if joint pain worsens. If worsens, refer.
- While waiting for appointment:
 - If pain in hands or feet, refer to occupational therapist, if available, and if pain in other joints, refer to physiotherapist, if available.
 - Assess and advise on chronic pain →61.

Yes

Acute gout likely
→152.

Yes

Sprain/strain likely

- Rest and elevate joint.
- Apply ice and a pressure bandage.
- Give **paracetamol** 1g 4-6 hourly (up to 4g in 24 hours) as needed for up to 5 days. If no response, give **ibuprofen** 400mg 8 hourly with food as needed for up to 7 days (avoid if peptic ulcer, asthma, hypertension, heart failure, kidney disease).
- If available, arrange physiotherapy.
- Advise to mobilise joint after 2-3 days, if not too painful.
- Review after 1 week: if no better, arrange x-ray and doctor review.